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TCGA-FY-A3R9-01A-PR Redacted



Patient Results

Surgical Pathology (Copath) Final

Path Report

SURGICAL PATHOLOGY REPORT

ACCESSION NO.

Final Diagnosis(es).

(A) Thyroid, thyroidectomy: Papillary thyroid carcinoma, multifocal, bilateral (see Synoptic report, below).

(B) Lymph node, pretracheal, excision: One lymph node, negative for neoplasm.

Synoptic Report:

Procedure: Total thyroidectomy.

Specimen Integrity: Fragmented.

Specimen Size:

Right Lobe: 4.7 x 2.0 x 1.3 cm.

Left Lobe: 3.9 x 2.2 x 1.6 cm.

Isthmus plus or minus Pyramidal Lobe: 1.5 x 1.5 x 0.8 cm.

Specimen Weight: 9.9 gm.

Tumor Focality: Multifocal - bilateral.

Tumor Laterality:

Right lobe (3 mm single focus).

Left Lobe: Multifocal - maximum size 7 mm).

Isthmus: 5 mm focus.

Tumor Size:

Greatest dimension: 0.7 cm (multiple separate foci, bilateral, including isthmus).

Histologic Type: Papillary carcinoma

Variant: Classical (usual) (60%).

Follicular variant (40%)

Architecture:

Classical (papillary) (60%)

Follicular (40%)

Cytomorphology: Classical

Histologic Grade: G2: Moderately differentiated.

Margins: Margin uninvolved by carcinoma (within 1 mm).

Tumor Capsule: Partially encapsulated.

Tumor Capsular Invasion: Present, minimal extent.

Lymphovascular Invasion: Not identified.

Perineural Invasion: Not identified.

Extrathyroid Extension: Not identified.

Second Tumor:

Tumor Laterality: Right lobe, left lobe and isthmus.

Tumor Size:

Greatest Dimension: 0.7 cm.

Additional Dimensions: 0.5 x 0.3 cm.

DIAGNOSIS OF PAPILLARY CARCINOMA CONVEYED TO

The gross description and all microscopic slides have been reviewed and interpreted by the undersigned pathologist.

Requested By:

Printed from: Default

Criteria	Yes	No
Diagnosis Discrepancy	X	
Primary Tumor Site Discrepancy		X
IPAA Discrepancy		X
Prior Malignancy History		X
Qual/Synchronous Primary Noted		X
Case is (circle) QUALIFIED DISQUALIFIED		
Reviewer Initials	Date Reviewed: 3/31/12	

Hurthle cell on clinical hx was shown to be papillary carcinoma on bx. ju

Frozen section consisted of follicular variant as indicated on CQCF.

ju
3/31/12

PATH ICD-O-3
carcinoma, papillary, thyroid

8260/3

Site: thyroid

C73.9

3-30-12
RD

CQCF: carcinoma, papillary,
follicular variant
8340/3

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Electronically Signed

Specimen(s) received:

A: Total thyroid bilateral lobes

B: Pre-tracheal lymph node

Clinical History:

Hurthle cell carcinoma

(A) short - superior left lobe, long - lateral right lobe.

Intraoperative Consultation:

Time Received: 1

Time Reported:

FSB1: Pretracheal lymph node: No carcinoma (1 block).

MD:

Gross Description:

(A) (total thyroid with bilateral lobes)

Specimen components and dimensions: The right lobe is 5.2 grams and measures 4.7 x 2.0 x 1.3 cm. The isthmus is 0.9 grams and measures 1.5 x 1.5 x 0.8 cm. The left lobe weighs 4.7 grams and measures 3.9 x 2.2 x 1.6 cm.

Inking Scheme: The specimen is inked as follows: right black, left blue, isthmus red.

Size, appearance, and location of tumor: Within the left lobe are three separate masses, all well circumscribed, pale gray, soft and fleshy with focal hemorrhage. Beginning superior, they measure 0.6, 0.7, and inferiorly measures 1.0 cm in greatest dimension. The right lobe has a single, central, indurated, gray-white nodule that is 0.5 cm in greatest dimension. The isthmus has two solid, gray-white nodules, both 0.4 cm in greatest dimension.

Number of tumor nodules: Six, three in the left lobe, one in the right, and two in the isthmus.

Capsular penetration: The two from the superior pole of the left lobe are difficult to discern. All others appear within the parenchyma and no extracapsular extension.

Soft tissue extension: None identified.

Other findings: None.

Uninvolved parenchyma: Red-brown, soft, lobulated tissue.

Blocks submitted: Entire right lobe superior to inferior in cassettes A1-A6. The isthmus is bisected and submitted in cassette A7, the left lobe is entirely submitted in sequence from superior to inferior in cassettes A8-A12.

(B) (pretracheal lymph node - FS) Received fresh for frozen section evaluation is a 0.8 x 0.8 x 0.2 cm, tan-pink, ovoid, rubbery tissue fragment that is bisected and submitted entirely for frozen section evaluation in cassette FSB1.

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Microscopic Description:

Complete microscopic evaluation has been performed.

Appropriately reacting controls have been performed and evaluated for all stains on this case as required.

Histopathology has a list of IHC antibodies that are regulated as analyte specific reagents (ASR's). These assays were developed and their performance characteristics determined by the Histopathology Laboratory in the Department of Pathology at the University of California, San Francisco. They have not been cleared by the US Food and Drug Administration. The FDA has determined that such clearance or approval is not necessary for the ASR's. These tests are not investigational and are used in standard clinical care. In cases where Immunohistochemistry testing is performed, the following antibodies and their respective clones may be used to determine therapy for the patient: EGFR(31G7), ER(SP1), PR(1E2), Her2neu(4B5), CD117(Poly), CD20(L26).

Unless otherwise stated in the report, all tissue tested for ERPR by IHC, Her2 by IHC and/or HER2 by FISH have been fixed as per ANP.22998 for a minimum of 6 hours and a maximum of 48 hours.

ER, PR, Ki-67, p53 are reported as a semi-quantitative percentage of positively stained nuclei. Her-2/neu and EGFR are scored as follows: No staining at all is scored as (0), weak, incomplete membrane staining in any proportion of cells is scored as (1+), less than strong but complete staining in any proportion of cells or complete strong staining in less than 30% of cells is scored as (2+), and strong complete staining in more than 30% of cells is scored as (3+). All studies are performed on tissue fixed in 10% neutral buffered formalin and embedded in paraffin unless otherwise stated in the report.